

Condition	Hospitalisation evidence	Severe / fatal evidence	Analysis role	Evidence refs	Key rationale
Diabetes	Supported	Supported	CORE	A, B, D, E, F, G	Consistent signals for hospitalisation and mortality
Hypertension	Supported	Limited	CORE	A, B, C, D, E	Strong hospitalisation signal; limited severe/fatal evidence
Chronic kidney disease	Supported	Supported	CORE	A, B, D, E, G	Largest risk signal; evidence chiefly from Brazil
Liver disease / hepatopathy	Insufficient	Limited	SECONDARY	E	Outcome-specific death signal in Brazilian national data
Autoimmune disease	Limited	Insufficient	SECONDARY	E	Limited association with hospital admission
Haematologic disease	Insufficient	Insufficient	NOT PRIORITISED	G	Sparse and heterogeneous prognostic evidence
Peptic / acid-peptic disease	Insufficient	Insufficient	NOT PRIORITISED	G	Very limited prognostic evidence

Evidence categories: Supported = consistent evidence of association; Limited = suggestive, outcome-specific, or sparse evidence; Insufficient = little or no prognostic evidence.

Operational prioritisation framework informed by a focused review; not a formal GRADE assessment. Evidence references are coded A through G.